



# Recognising delirium: look for change from usual

Help staff recognise a new or fluctuating change that may be delirium, including quieter presentations that are easily missed.

## Look for an acute or fluctuating change

- Ask whether alertness, attention, thinking, perception or behaviour has changed over hours or days.
- Notice fluctuation: a person may appear clearer at one contact and substantially different at another.
- Treat a credible concern from family, carers or staff who know the person as clinically important information.

uptake. Final named human clinical approval is required before use in patient care. Independent resource; not an official University of Edinburgh or NHS publication.

## Do not look only for agitation

- Delirium may present with restlessness, fear, suspiciousness, hallucinations or disorganised speech.
- It may instead present with drowsiness, withdrawal, slow responses, reduced movement, poor concentration or reduced appetite. This quieter pattern is often called hypoactive delirium.
- Mixed presentations can move between quieter and more active states.

## Establish the usual baseline

- Find out the person's usual cognition, communication, alertness and level of function.
- Use more than one source where possible: the person, family or carers, care records, referral information and staff observations.
- Check that hearing, vision, language or communication difficulties are not obscuring the assessment.

## Record and assess

- Document the observed change, its timing, fluctuation and the source of baseline information.
- If indicators of delirium are present, arrange assessment with the appropriate tool and a clinical evaluation.
- If delirium and dementia are difficult to distinguish, address possible delirium first.

Safety box - act on sudden change: New confusion or a marked change in alertness may signal serious acute illness and needs prompt clinical assessment. A detection tool supports recognition; it does not by itself make the final diagnosis. Follow the setting's emergency response if the person is acutely unstable or difficult to rouse. NICE recommends CAM-ICU or ICDSC rather than 4AT in critical care or the recovery room after surgery.

## Verified official sources

- [NICE CG103: recommendations](#)
- [SIGN 157 on Right Decisions: detecting delirium](#)
- [Official 4AT user guide](#)
- [NHS inform: delirium](#)

Use and review: UK-focused general education. This resource supports but does not replace clinical assessment, individual advice or current local guidance. Prepared from source materials curated by Professor Alasdair MacLulich, University of Edinburgh; AI-assisted production. Professor MacLulich led development of the 4AT and reports no financial interest in its